جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : سعاد محمود زاهر شريف
المحافظة : القاهره **الحي / المدينة :** بولاق ابوالعلا **العنوان :** الجيزه
رقم البطاقة : 26310120104168 **رقم التليفون :** 01010380685 **تاريخ تسجيل الإستمارة :** 17-10-2026
العمر(تاريخ الميلاد) : 12-10-1963 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: سعاد محمود زاهر شريف
National ID: 26310120104168

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Supportive and palliative Treatment**

التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية



سعاد
محمود زاهر شريف
٣ش أحمد سرور-ش أحمدعثمان-الكنيسة
بولاق الدكرور - الجيزه

١٩٦٣/١/٢

٢٦٣١٠١٢٠١٠٤١٦٨

II5436419

٢٠٢٠/٠٢

ربة منزل

أنثى مسلمة متزوجة
كمال احمد محمد عثمان

١ البطاقة سارية حتى ٢٠٢٧/٠٢/١٠

National ID – Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

Medical No. :	**9313**
Patient's Name:	**Soad Mahmoud Zaher Sherif**
Date:	**2026-09-16**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

Known case of CA breast. No available previous DICOM images for comparison.

I- CHEST:

- Right breast lesion seen measuring 5 x 2.2 cm for sono-mammography.
- Diffuse lung mosiac perfusion seen yet no sizable pulmonary nodules or masses seen or cavitary lesions.
- No consolidation or collapse.
- Multiple mediastinal lymph nodes seen at paratracheal and prevascular and retrocaval lymph nodes largest measuring 1.5 x 1.3 cm seen at right para tracheal region.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.
- Bilateral elastofibroma dorsi.

II- ABDOMEN AND PELVIS:

- Enlarged liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Relatively small sized right kidney showing cortical focus of calcification and right renal cortical cyst (old surgical interference). Normal function. No stones or backpressure changes.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Right adrenal hypodense lesion measuring 2.7 x2.6 cm likely adenoma for MR assessment.
- Normal C.T appearance of the spleen, pancreas, left adrenal gland, aorta and IVC.

CT Scan
[[DOCTYPE:CT_SCAN]]

Medical No. : 9313
Patient's Name: Soad Mahmoud Zaher Sherif
Date: 2026-09-16
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

- Multiple gastro-esophageal junction, porta hepatis, para-aortic, precaval, peri pancreatic and bilateral external iliac lymph nodes are seen largest measuring 28 x 13 mm at the precaval region.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- IUCD is seen within the pelvis.
- Normal appearance of the rest of pelvic organs with no pelvic masses or collections.
- No ascites.
- Dorso-lumbar spondylotic changes.
- bilateral sacroilitis

Opinion:

- **Mediastinal and abdominal enlarged lymph nodes for PET-CT assessment.**
- **No pulmonary or hepatic metastatic lesions.**

on Compression
C.T CAP → SD

Dr.sherif Wagdy

CT Scan
[[DOCTYPE:CT_SCAN]]

GUSTAVE ROUSSY
INTERNATIONAL
EGYPT

Name : SOUAAD MAHMOUD ZAHER	**Date : 30-12-2025 02:48**
Branch : Portable	**Medical No. : GRI-26310120104168**
Ref.Physician Prof :	**Ins.Company :**

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

Known case of CA breast. No available DICOM images for comparison.

I- CHEST:

- Right breast lesion is seen measuring 5.1 x 2.3 cm for sono-mammography.
- Few bilateral scattered tiny subpleural pulmonary nodules largest measuring 5.8 mm at the right lower lobe.
- No pulmonary masses or cavitary lesions.
- No consolidation or collapse.
- Few paratracheal and prevascular lymph nodes largest measuring 19 x 9.7 mm.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.
- Bilateral elastofibroma dorsi.

II- ABDOMEN AND PELVIS:

- Enlarged liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Relatively small sized right kidney showing cortical focus of calcification and right renal cortical cyst. Normal function. No stones or backpressure changes.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Right adrenal lesion is seen measuring 30 x 21 mm.
- Normal C.T appearance of the spleen, pancreas, left adrenal gland, aorta and IVC.
- Multiple gastro-esophageal junction, porta hepatis, para-aortic, precaval, peri pancreatic and bilateral external iliac lymph nodes are seen largest measuring 25 x 15 mm at the precaval region.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- IUCD is seen within the pelvis.
- Normal appearance of the rest of pelvic organs with no pelvic masses or collections.

[[DOCTYPE:CT_SCAN]]


جمهورية مصر العربية
بطاقة تحقيق الشخصية
سعاد
محمود زاهر شريف
٣ش احمد سرور-ش احمد عثمان-الكنيسة
بولاق الدكرور - الجيزه
٢٦٣١٠١٢٠١٠٤١٦٨
١٩٦٣/١/١٢
II5436419

DMS


٢٦٣١٠١٢٠١٠٤١٦٨ ٢٠٢٠/٠٢
ربة منزل
أنثى مسلمة متزوجة
كمال احمد محمد عثمان
البطاقة سارية حتى ٢٠٢٧/٠٢/١٠

National ID – Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

Patient's Name:	souad mahmoud zaher shiref
Date:	2026-04-29
Referred by:	Prof.Dr. ct cap iv
Hospital:	Goustave Roussey International Egypt

2976

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

Known case of CA breast. Compared to last study dated 30-12-2025 the current study reveals

I- CHEST:

- No appreciable time interval changes regarding the size and shape of the previously seen right breast lesion seen measuring 5.5 x 2.2 cm for sono-mammography.
- Still noted diffuse lung mosiac perfusion seen yet no sizable pulmonary nodules or masses seen or cavitary lesions.
- No consolidation or collapse.
- Still seen multiple mediastinal lymph nodes seen at paratracheal and prevascular and retrocaval lymph nodes largest measuring 3 x2.1 seen at right para tracheal region.
- Bilateral axillary centimetric and sub centimeter lymph nodes seen
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.
- Bilateral elastofibroma dorsi.

II- ABDOMEN AND PELVIS:

- Enlarged liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Relatively small sized right kidney showing cortical focus of calcification and right renal cortical cyst (old surgical interference). Normal function. No stones or backpressure changes.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- No appreciable time interval changes regarding the size and CT appearance of the previously noted right adrenal hypodense lesion measuring 2.7 x2.6 competed to 3x2 cm , likely lipid poor adrenal adenoma.

CT Scan
[[DOCTYPE:CT_SCAN]]

Patient's Name:	souad mal moud zaher shiref
Date:	2026-04-29
Referred by:	Prof.Dr. et cap iv
Hospital:	Goustave Roussey International Egypt

- Normal C.T appearance of the spleen, pancreas, left adrenal gland, aorta and IVC.
- Stationaty course regarding the previously seen multiple gastro-esophageal junction, porta hepatis, para-aortic, precaval, peri pancreatic and bilateral external iliac lymph nodes are seen largest measuring 25 x 15 mm at the precaval region.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- IUCD is seen within the pelvis.
- Normal appearance of the rest of pelvic organs with no pelvic masses or collections.
- No ascites.
- Diffuse dorso-lumber advanced osteoarthritic changes with multilevel fusion osteopathy otherwise no CT evidence of osteolytic or scerotic lesions seen
- bilateral sacroiliitis

Opinion:

Known case of CA breast showing rather stationary course since last study.

Dr.sherif Wagdy

CT Scan
[[DOCTYPE:CT_SCAN]]

GUSTAVE ROUSSY
INTERNATIONAL

NAME: soad mahmoud
DATE;29/4/2026

Duplex Examination of the venous systems of both lower limbsrevealed: -

Deep Venous System: -

- *The femoral (common & superficial), popliteal & infrapopliteal veins of the examined lower limb all are Patent, totally compressible with normal color filling & preserved respiratory variation.*
- *No evidence of D.V.T at time of scan.*

Superficial Venous System: -

- *Patent long & short saphenous veins with no evidence of superficial thrombophlebitis at time of scan.*

OPINION: -

No evidence of acute D.V.T or superficial thrombophlebitis at time of scan

Mild interstitial edema of the examined limbs

Best regards
Dr. NERMEEN RAMADAN FEBR. MD

dupplex
[[DOCTYPE:OTHER_DUPPLEX]]

Cairo University
National Cancer Institute
Department of Diagnostic and
Interventional Radiology

جامعة القاهرة
المعهد القومي للأورام
قسم الأشعة التشخيصية و الداخلية

Exam Information:

Patient Name: SOAD MAHMOUD ZAHER	*Date of birth:* 12/10/1963
Arabic Name: سعاد محمود زاهر	*Patient Gender:* F
Patient RIS ID: 384621 *Patient HIS ID:* 10110226006419	*Exam Date:* 11/05/2026
Written by: Dr. Hoda Fathy	Approved by: Dr. Hoda Fathy

^{99m}Tc MDP Bone Scan Report (11/05/2026)

* Whole Body Anterior and Posterior Planar Views Acquired 2hrs. Post I.V Injection of 20mCi of ^{99m}Tc MDP

Revealed:

- No definite evidence of active osseous deposits could be seen over the surveyed skeleton.
- Degenerative changes are noted over the dorso-lumbar vertebrae.
- Arthritic changes are noted over both shoulder and both knee joints.
- Both kidneys are visualized.

Conclusion:

- No definite evidence of active metastatic osseous deposits could be seen all over the surveyed skeleton.

Much Obliged

Hoda Yasser

Randa Koura, MSc

Bone Scan
[[DOCTYPE:BONE_SCAN]]

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

MDT FORM 14/5/2025

Name	سعاد محمود زاهر	MRN	٨٢٢٣١١٩
Age	٦١ سنة	ID	٢٩٢٠١٠١٠٤٠١٦
Menopausal status	Post.	Phone	٠١١١٢٤٦٥٣٤
NRS SCORE		Marital status	٢٩٧٠٤٢٣٠١٠٢
BMI		Family history	٢٢٢٢٢٢
CO morbidity	D.M + HTN	REASON OF MDT	Re-Dt

Clinical Data

BX: IDC

ER:	PR:	HER 2:	KI67:
8/8	7/8	-ve.	

R

4

Sonomammography:
MBC → 6 med. LN
Contra lateral axilla.

Mets Workup:
Femara 12/2024
PET CT 5/2024: breast Lesion.
med. LNs

TNM

Recommendations:
For your recommedation
Compare petct to CT- 5/2024
→ PD of mediastinal LN sgs.

MDT OUTCOME & PLAN
① 1st Fu 2nd line: Faslodex / Ibrance
② Zometa / Gm.

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. رضا الدباي د. زينب [illegible]					

SMC-DSILF-MDT-001
Revision no 0
Page 1 of 1
Effective date: 3/2026

MDT Form
[[DOCTYPE:MDT_FORM]]

Patient Name	: [illegible]		
Patient No.	: 26310120104168	Request No.	: 13005527
Path No.	: 3128/24	Request Date	: 28/08/2024
Sex	: Female	Verification Date	: 10/09/2024
Age	: 61 Y	Financial Category	: [illegible]
Referring Doc.	:	Department	: Out Patient
		IHC No.	:
		Registered by	: Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right breast mass.
Nature of Specimen: Tru-cut biopsy.

Technique:
- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (6P1), Her2 clone (4B5), & KI67 clone (30-9).

Results:
* IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 60% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 60% of tumor cells (score 8/8).
- Her2: Faint incomplete membrane reaction in >10% of the tumor cells (score 1+).
- KI67: 15% nuclear reaction of tumor cells.

Conclusion:
Right breast mass, Tru-cut biopsy:
Invasive duct carcinoma, Grade II.
IPT:
- ER: Positive (score 8/8).
- PR: Positive (score 7/8).
- Her2: Score 1+.
- KI67: Positive in 15% of tumor cells.

* Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1,

Verified by :

Room No. (4) - First Floor-Building No. 582- Nile Corniche - Mar El Qadema - Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

Patient Name		Request No.	13005527
Patient No.	26310120104168	Request Date	28/08/2024
Path No.	3128/24	Verification Date	10/09/2024
Sex	Female	Financial Category	
Age	61 Y	Department	Out Patient
Referring Doc.		Registered by	Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

>1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 6.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within ≤ 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within ≤ 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10%of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Dr. Rana Fathy,
Dr. Sagda Elkady,

Verified by :

Printing Date : 19/11/2024 16:08:56 Page 2 of 2 Printed By : Manar mohamed yousr

Immunohistochemistry
[[DOCTYPE:IHC]]

Patient Name	:	Request No.	: 13005527
Patient No.	: 26310120104168	Request Date	: 28/08/2024
Path No.	: 3128/24	Verification Date	: 02/09/2024
Sex	: Female	Financial Category	:
Age	: 61 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Right breast mass.
Nature of Specimen: Tru-cut biopsy, collected at 27/08/2024.

Gross Examination:
- Four grayish white tissue cores up to 1 cm, totally submitted.

Microscopic Examination:
- Histological type: Invasive duct carcinoma.
- Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
- Ductal carcinoma in situ: No.
- Perineural invasion: No.
- Angioinvasion: No.

Diagnosis:
Right breast mass, Tru-cut biopsy:
Invasive duct carcinoma, Grade II.
For further IHC (ER, PR, Her2, Ki67).

Signature
Dr. Rana Fathy.
Dr. Sagda Elkady.

Verified by :

Room No. (4) - First Floor-Building No. 987, Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

PATIENT NAME: SOAD MAHMOUD ZAHER SHREIF
GENDER: Female
DATE : 16-May-2025 11:06 AM

PATIENT ID: 10124251095
AGE: 61 y
ORDER: 3379514

WHOLE BODY FDG PET-CT EXAMINATION

Clinical background:

- 61-year-old female patient.
- Presented with bony pain.
- Given history of right breast cancer on hormonal treatment. No available pathological data or previous studies.
- Weight = 115 Kg.
- BL glucose level = 128 mg/dl.

Reason of study:

- Metastatic work up.
- Radiopharmaceutical:
- F-18-FDG 8 mCi Injected.

Technique:

- Approximately one hour after IV injection of the radiopharmaceutical, whole body PET-CT examination from the skull to mid-thigh was performed with arms extended above head.
- Non-contrast CT was acquired for the purpose of attenuation correction and anatomical localization.
- No IV contrast was given.
- Data was processed and displayed according to department protocol.
- Calculation of SUVmax was done at region of interest.
- Reference hepatic activity (SUV max)~2.51.

Findings:

Reviewing PET, CT and fused PET-CT images:-

I- Breasts and axillary regions:

- **Right breast** inner domain metabolically active (FDG avid) irregular soft tissue mass with related air locull is seen measuring about 2 X 6.5 cm with SUV max~4.15 seen inseparable from the underlying pectoral muscle.

- Multiple right axillary lymph nodes (levels I and II), showing no significant FDG uptake.
- Few non FDG avid bilateral supraclavicular lymph nodes, the largest on the right side measuring about 0.7 x 1.1 cm for correlation with sonographic findings.
- No metabolically active (FDG avid) contralateral breast masses or axillary lymph nodes.

II. Head and neck:

- Diffuse symmetrical tracer uptake is seen along the examined brain with no metabolically active (FDG avid) lesions could be detected.
- No metabolically active (FDG avid) cervical lymph nodes.
- No metabolically active (FDG avid) neck masses.
- Physiological symmetrical tracer uptake at salivary glands with no metabolically active (FDG avid) lesions.
- Normal appearance of the aerodigestive air column with no parapharyngeal masses.
- Normal appearance of the vocal cords, both pyriform sinuses and aryepiglottic folds. No glottic, supra- or infraglottic masses.
- Intact laryngeal skeleton.
- Enlarged thyroid gland with heterogeneous texture showing no FDG avid nodules.

III. CHEST:

- No metabolically active (FDG avid) pulmonary nodules.
- Few metabolically active mediastinal lymph nodes, the largest and most active one is noted at subcarinal group measuring about 1.9 x 3.2 cm with SUV max about 3.97.
- No metabolically active pleural effusion or masses.
- Enlarged cardiac size. No pericardial effusion detected.
- Low grade metabolically active bilateral elastofibroma dorsi.

IV. ABDOMEN AND PELVIS:

- The liver is average in size with homogenous CT texture. No definite CT evidence of metabolically active (FDG avid) focal lesions or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Low grade metabolically active right adrenal gland well defined lesion measuring about 2 x 3 cm with SUV max about 2.26 and +10 HU mostly adenoma.
- Small sized right kidney showing normal FDG excretion, lower pole parenchyma calcific focus about 6 mm and small non FDG avid cortical cyst.
- Normal appearance of the pancreas, spleen, left kidney and left suprarenal (on non contrast CT basis) with No FDG avid focal lesions.

- No metabolically active abdominal or pelvic lymphadenopathy.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- No ascites.
- Bulky anteverted uterus showing contraceptive device within.
- Small fat containing para umbilical hernia.

V. Scanned axial and proximal appendicular skeleton:

- No metabolically active (FDG avid) osseous lesions.
- D6 and D9 lucent metabolically inactive lesions.
- Dorsolumbar spondylosis.

OPINION:

A reported case of right breast cancer showing:

- Right breast metabolically active (FDG avid) mass lesion.
- Metabolically active mediastinal lymph nodes of indeterminate nature for follow up.
- No definite metabolically active (FDG avid) pulmonary, hepatic or osseous metastatic deposits.

Reported By,
Dr. Fifi Samy, MSc
Dr. Mohamed Mohsen, MD
Prof. Huda Fathy, Nuclear Medicine Consultant 17-May-2025 05:27 PM